Reducing Avoidable ER Visits

Financial analysis of three options for Good Health Physicians

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The business case

525

ER visits per 1,000 members today

450

Target within 3 years

$1,200

Cost of each ER visit

5,000 → 6,000

Members, Year 1 to Year 3

Good Health carries the financial risk, so every avoided ER visit saves money. But each option also has its own cost.

Options evaluated

1. Hire an additional practitioner
2. Build an urgent care center
3. Add a triage nurse phone line

Methodology and key assumptions

Net savings = avoided ER visits × $1,200 − (running cost + replacement visit costs)

Option	ER reduction	Running cost	Replacement visits
1. Practitioner	10% in Year 1, +5% in Year 2 (15% cumulative)	$90K salary, +3% a year	50% of avoided visits become a $75 office visit
2. Urgent care	15% × 40% of members who can reach it = 6%	$115K, +5% a year	All avoided visits become a $150 urgent care visit
3. Nurse line	10% in Year 1, +5% in Year 2 (15% cumulative)	$10K set-up (Year 1), $5K marketing and $20K overhead a year	30% to urgent care ($150), 40% to primary care ($75), 30% none

Everything is built in Excel with cell references, so changing any input updates every result.

Model: all assumptions in one sheet

General	Year 1	Year 2	Year 3
Members	5000	5500	6000
Baseline of ER visit/1000	525	525	525
Cost per visit ($)	1200	1200	1200
Target per visit/1000	450	450	450

Option 1

Label	Year 1	Year 2	Year 3
Practitioner salary ($)	90000	92700	95481
Annual salary inflation (%)	3%	3%	3%
ER reduction (%)	10%	5%	0%
Share of avoided visits replaced by office visit (%)	50%	50%	50%
Cost per office visit ($)	75	75	75

Option 2

Label	Year 1	Year 2	Year 3
General Operating Cost($)	115000	120750	126787.5
Annual increase in operational cost	5%	5%	5%
Reduction of ER visit	15%	15%	15%
Share of members who can reach	40%	40%	40%
Urgent care visit cost($)	150	150	150

Option 3

Label	Year 1	Year 2	Year 3
New Phone Line set-up cost($)	10000	0	0
Marketing material cost/year ($)	5000	5000	5000
Overhead cost/year ($)	20000	20000	20000
Reduction in ER visit	10%	5%	0%
Share redirected to urgent care (%)	30%	30%	30%
Share redirected to primary care (%)	40%	40%	40%
Share with no offsetting cost (%)	30%	30%	30%
Urgent Care Cost ($)	150	150	150
Office Visit cost($)	75	75	75

Blue cells are inputs. The option sheets link to these cells, so nothing is typed into a calculation.

Model: Option 3 calculations

A	B	C	D	E
	Year 1	**Year 2**	**Year 3**	**3- Yr Net**
Members	5000	5500	6000	
Baseline ER visits	2625	2887.5	3150	
Cumulative reduction %	10%	15%	15%	
Avoided ER visits	262.5	433.125	472.5	
Gross savings	315000	519750	567000	1401750
Operating cost	35000	25000	25000	
Offsetting cost, urgent care	11812.5	19490.625	21262.5	
Offsetting cost, primary care	7875	12993.75	14175	
Offsetting cost, no other visit	0	0	0	
Total Offsetting costs	19687.5	32484.375	35437.5	
Total cost	54687.5	57484.375	60437.5	172609.375
Net savings	260312.5	462265.625	506562.5	1229140.625
ER visits per 1,000 after the program	472.5	446.25	446.25	**Target met**

- Avoided visits = baseline visits × cumulative reduction
- Gross savings = avoided visits × $1,200
- Total cost = operating cost + replacement visit costs
- Net savings = gross savings − total cost
- Last row checks Year 3 against the 450 target
- Options 1 and 2 use the same layout

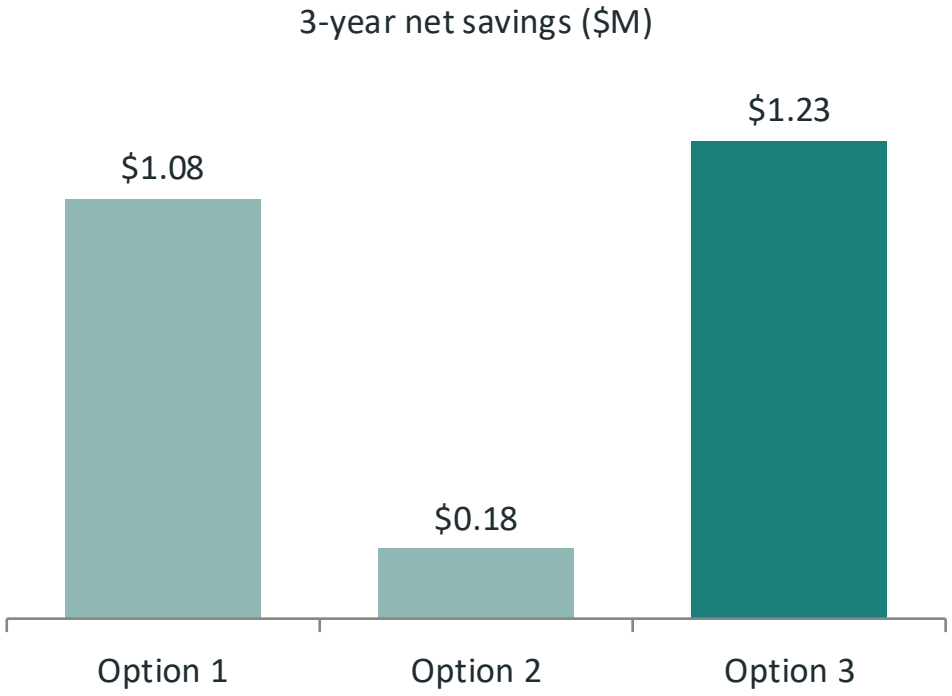
Findings: summary of the three options

	Option 1: Additional practitioner	Option 2: Urgent care center	Option 3: Triage nurse line
Gross savings	1,401,750.00	623,700.00	1,401,750.00
Total cost	321,985.69	440,500.00	172,609.38
Net savings	1,079,764.31	183,200.00	1,229,140.63
Year 3 ER rate	446.25	493.50	446.25
Target met?	**Yes**	**No**	**Yes**

- Option 3 has the highest net savings: $1.23M over 3 years
- Option 1 follows at $1.08M, but costs almost twice as much
- Option 2 saves only $183K and misses the target

Option 3 gets the same ER reduction as Option 1 for about half the cost.

Findings: savings and the 450 target



Year 3 ER visits per 1,000 (target 450)

Option 1	**446**	Target met
Option 2	**494**	Missed
Option 3	**446**	Target met

Recommendation: Option 3 (nurse line)

$1.23M

3-year net savings

446

ER visits per 1,000, under the 450 target

$173K

3-year cost, vs $322K for Option 1

Risks to manage

- Members must actually use the line, so keep the marketing budget and track call volume
- 446 leaves only about 4 visits per 1,000 of margin under 450
- If the risk is too high, Option 1 is the safer alternative, at about $150K more over 3 years